

Notice from the National Commission for Data Protection regarding Bill No. 8331 amending the amended law of August 1, 2018 on the mandatory declaration of certain diseases within the framework of public health protection and regarding the draft Grand-Ducal regulation specifying the modalities and conditions for the establishment of the electronic vaccination record.

Deliberation No. 9/AV4/2024 of February 23, 2024.

1.

In accordance with Article 57.1.e) of Regulation (EU) 2016/679 of the European Parliament and of the Council of April 27, 2016 on the protection of natural persons with regard to the processing of personal data and on the free movement of such data, and repealing Directive 95/46/EC (General Data Protection Regulation) (hereinafter referred to as the "GDPR"), to which Article 7 of the law of August 1, 2018 on the organization of the National Commission for Data Protection and the general regime on data protection refers, the National Commission for Data Protection (hereinafter referred to as the "National Commission" or the "CNPD") "advises, in accordance with the law of the Member State, the national parliament, the government, and other institutions and bodies regarding legislative and administrative measures relating to the protection of the rights and freedoms of natural persons with regard to processing."

Furthermore, Article 36.4 of the GDPR states that "[m]ember states consult the supervisory authority in the context of the preparation of a legislative proposal to be adopted by a national parliament, or of a regulatory measure based on such a legislative measure, which relates to processing."

The European Data Protection Board (hereinafter referred to as the "EDPB") further specifies that for any limitation on the rights of data subjects adopted at the level of Member States, the data protection supervisory authority must be consulted before the adoption of such a measure by the national parliament.

2.

By letter dated October 20, 2023, the Ministry of Health invited the National Commission to express its opinion on Bill No. 8331 amending the amended law of August 1, 2018 on the mandatory declaration of certain diseases within the framework of public health protection (hereinafter referred to as the "draft bill"). Furthermore, the said Ministry referred the CNPD to the EDPB regarding the draft Grand-Ducal regulation specifying the modalities and conditions for the establishment of the electronic vaccination record.

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Letter dated October 25, 2023, to notify the draft Grand-Ducal regulation specifying the modalities and conditions for the establishment of the electronic vaccination record (hereinafter referred to as the "draft Grand-Ducal regulation"). The draft Grand-Ducal regulation is enacted in execution of the future Article 46/s of the amended law of August 1, 2018, on the mandatory declaration of certain diseases within the framework of public health protection and the implementation of an electronic vaccination record (hereinafter referred to as the "future Article 46/s of the amended law of August 1, 2018, on the mandatory declaration of certain diseases"), as inserted after Article 4 of said law by Article 2 of the aforementioned draft law.

3. **Although the National Commission does not wish to challenge the usefulness of establishing an electronic vaccination record (hereinafter referred to as the "digital record") intended to replace the physical record, known as the "yellow card," for the benefit of the patient as well as for improving the efficiency and transparency of national vaccination campaigns, it emphasizes that sufficient

guarantees regarding the respect for the fundamental principles of the right to the protection of personal data must be implemented.**

4. **In general, the CNPD welcomes in this context that the draft law and the draft Grand-Ducal regulation intend to provide a legal basis for the processing of data carried out in the context of the establishment of the digital record and its subsequent use. It notes that the digital record aims, among other things, to improve patient care by enabling the healthcare provider to deliver quality healthcare, on one hand, and allowing the patient to make an informed decision regarding vaccination, on the other hand, while granting the State the ability to manage vaccine stocks and monitor and evaluate the vaccination coverage of recommended vaccines. Its implementation could therefore correspond to a public interest mission in accordance with Article 6.1.e) of the GDPR.**

Article 6.3 of the GDPR provides in this context a specific constraint related to the lawfulness of data processing necessary for the fulfillment of a public interest mission or related to the exercise of public authority vested in the data controller. In this case, the basis and notably the purposes of data processing must be specifically provided either by Union law or by the law of the Member State to which the data controller is subject, and this basis must meet "a public interest objective and be proportionate to the legitimate objective pursued."

2. **Article 1 of the draft law aims to replace the title of the amended law of August 1, 2018, on the mandatory declaration of certain diseases within the framework of public health protection with the following title: "Amended Law of August 1, 2018, on the Mandatory Declaration of Certain Diseases within the Framework of Public Health Protection and the Implementation of an Electronic Vaccination Record."**

3. **See the explanatory memorandum of the draft law.**

4. **See the explanatory memorandum of the draft law, as well as the purposes mentioned in the future Article 4ù/s of the amended law of August 1, 2018, on the mandatory declaration of certain diseases, as well as the commentary on the articles, ad Article 2.**

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5.
Furthermore, insofar as part of the data processed through the digital notebook relates to the health of the patients concerned, these are to be classified as special categories of data, referred to as "sensitive data," within the meaning of Article 9 of the GDPR. Such processing requires specific protection and is subject to stricter requirements. The processing of sensitive data is, in fact, prohibited unless one of the conditions referred to in paragraph 2 of Article 9 of the GDPR is met. The National Commission believes that the data processing carried out through a digital notebook could fall under the public interest grounds in the field of public health referred to in Article 9.2.i) of the GDPR, provided that European Union law or national law provides for "appropriate and specific measures for the safeguarding of the rights and freedoms of the data subject."

6.
In this case, it is appropriate to commend the authors of the draft law and the draft Grand-Ducal regulation for having included clarifications regarding data protection in their respective texts. Nevertheless, the CNPD wishes to make remarks on certain specific points.

1.
Regarding the data controller

7.

The CNPD wishes to remind that in accordance with Article 4.7) of the GDPR, the data controller is "the natural or legal person, public authority, service, or other body which, alone or jointly with others, determines the purposes and means of the processing; where the purposes and means of such processing are determined by Union law or the law of a Member State, the data controller may be designated or the specific criteria for its designation may be provided for by Union law or the law of a Member State."

8.

With regard to the notions of data controller and processor, it follows from the guidelines of the EDPS on this subject that "[W]hen the data controller is specifically identified by law, this designation is decisive in defining who acts as the data controller. This presupposes that the legislator has designated as the data controller the entity that is truly able to exercise control."

9.

It should also be noted that the notion of data controller, even of joint responsibility, are functional notions in that they aim to allocate responsibilities in the processing of personal data.

5 EDPS, Guidelines 07/2020 on the concepts of data controller and processor in the GDPR, version 2.0, adopted on July 7, 2021, point 23.

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function of the actual roles played by the parties. Thus, they rely on a factual rather than formal analysis.⁶

10.

In this case, the future article 4b/s of the amended law of August 1, 2018, on the mandatory declaration of certain diseases provides that "[...] the National Agency for Shared Health Information shall make an electronic vaccination record available to patients, healthcare providers, and the health authority." In the commentary on article 2 of the draft law, the authors explain, after justifying the establishment of the digital record, that its availability will be entrusted to the National Agency for Shared Health Information (hereinafter referred to as the "eHealth Agency"), because "it is already responsible for managing the national electronic platform with the shared care record. This will allow the electronic vaccination record to benefit, for example, from a high level of security as already ensured in the context of the shared care record."

11.

The text of the draft law thus creates the impression that the eHealth Agency, established based on article 60ter of the Social Security Code and subject to the joint authority of the ministers responsible for Health and Social Security, would act alone as the data controller for the personal data processing carried out in the context of the implementation of the digital record. Nevertheless, the CNPD notes that clarifications in this regard can be found in the draft Grand-Ducal regulation, as well as in its commentary on the articles.⁷ From its reading, the following configuration emerges:

The eHealth Agency will be responsible for the data processing carried out by the very creation of the digital record for each natural person with a national identification number⁸ and this from the first encoding by the healthcare provider. Furthermore, pursuant to article 1(4) of the draft Grand-Ducal regulation, the eHealth Agency must proceed, upon receipt of a death certificate or the date of death from the State Information Technology Center, to archive the data of the digital record for the legal retention period.

⁶ Page 3 and points 21 and 52 of the aforementioned guidelines.

⁷ See for example the commentary on the articles, ad article 2: "As the electronic vaccination record is created on the national electronic platform for shared health information and healthcare providers

record data related to their patients' vaccinations as they currently do on the paper yellow card, this draft Grand-Ducal regulation formalizes this sharing of responsibilities by establishing joint responsibility between these two stakeholders. The same applies to data concerning the management of vaccine stocks recorded by the health authority, which may include personal data of the healthcare providers to whom vaccines are distributed."

8 The commentary on article 1 of the draft Grand-Ducal regulation specifies that "[i]n the long term, an electronic vaccination record should therefore be created for every resident and every non-resident affiliated with the National Health Fund."

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For the processing of data recorded by healthcare providers in the digital notebook, Article 2 (1) of the draft Grand-Ducal regulation provides that said providers act as joint controllers with the eHealth Agency within the meaning of Article 26 of the GDPR.

Finally, for the processing of data recorded by the health authority regarding the management of vaccine stocks, which may include personal data of healthcare providers, Article 2 (2) of the draft Grand-Ducal regulation provides that the health authority acts with the eHealth Agency as joint controllers within the meaning of the aforementioned Article 26 of the GDPR.

12. The CNPD nevertheless has difficulties linking the aforementioned processing operations carried out by the various actors to the purposes mentioned in the future Article 4ib/s (2) of the amended law of August 1, 2018, on the mandatory declaration of certain diseases. In other words, it believes that it is not clearly evident from the texts under review for which of the various purposes pursued the eHealth Agency, the health authority, as well as healthcare providers should be considered as responsible, or even as joint controllers of the processing carried out through the digital notebook.

13. Therefore, taking into account that the protection of personal data is a matter reserved for law by the Constitution, on the one hand, and that in such matters the essence of the normative framework must result from the law, including the purposes, conditions, and modalities under which less essential elements may be regulated by regulations and decrees issued by the Grand Duke, the National Commission recommends that the authors of the draft law clarify in the body of the text who is to be qualified as responsible, or as joint responsible parties for the processing, distinguishing between the different purposes to be achieved.

14. Finally, it should be noted that Article 26.1 of the GDPR states that joint controllers of processing must transparently define their obligations.

9 The future Article 4b;s (4) of the amended law of August 1, 2018, on the mandatory declaration of certain diseases indeed provides that it is the healthcare providers who "record at the time of vaccination and no later than within seventy-two hours of it in the electronic vaccination notebook the data referred to in paragraph (3) for any vaccination recommended by the State. [...]"

10 According to Article 2 of the current law of August 1, 2018, on the mandatory declaration of certain diseases, the director of health or his delegate is to be designated as the "health authority."

11 See commentary on the articles of the draft Grand-Ducal regulation, ad Article 2.

12 See Articles 31 and 37 of the Luxembourg Constitution. According to the jurisprudence of the Constitutional Court, Article 45, paragraph 2, of the Constitution indeed requires that in these matters, "the setting of the objectives of the implementing measures must be clearly stated, as well as the conditions to which they are, if applicable, subject. The orientation and framework of executive power must, in any case, be consistent, precise, and readable, the essence of the relevant provisions being

expected to be included in the law" (Constitutional Court, June 4, 2021, No. 166, Mém. An°440 of June 10, 2021, and Constitutional Court, March 3, 2023, No. 177, Mém. A, No. 127 of March 10, 2023).

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****National Commission for Data Protection Opinion****

****Regarding Bill No. 8331 Amending the Modified Law of August 1, 2018 on the Mandatory Declaration of Certain Diseases in the Context of Public Health Protection and Regarding the Grand-Ducal Regulation Specifying the Terms and Conditions for the Implementation of the Electronic Vaccination Record.****

****I. Regarding the Compliance with GDPR Requirements****

1. In order to ensure compliance with the requirements of the GDPR, particularly concerning the exercise of the rights of the data subject, the guidelines of the European Data Protection Board (EDPB) state that "it is appropriate to allocate responsibilities for compliance, as is evident from the use of the term 'respective' in Article 26, paragraph 1. This does not exclude that Union law or the law of a Member State may already establish certain responsibilities for each joint controller. If this is the case, the agreement between the joint controllers should also cover any additional responsibilities necessary to ensure compliance with the GDPR requirements that are not covered by legal provisions."

2. Paragraphs (3) and (4) of Article 2 of the draft Grand-Ducal Regulation specify, in this context, the modalities for exercising the rights of data subjects. The CNPD wishes to draw the attention of the authors of the draft Grand-Ducal Regulation to the fact that "[i]t is only a matter of defining, in their internal relations, which party is obliged to respond to the requests of the data subjects." Indeed, regardless of the terms of the agreement between joint controllers, the data subject may contact each of the joint controllers in accordance with Article 26.3 of the GDPR.

****II. Regarding Personal Data Collected****

3. In terms of compliance with the principles relating to the processing of personal data as provided by the GDPR, the CNPD reminds that personal data must be "collected for specified, explicit, and legitimate purposes, and not further processed in a manner incompatible with those purposes [...] (purpose limitation)" and that they must be "adequate, relevant, and limited to what is necessary in relation to the purposes for which they are processed (data minimization)."

4. The CNPD believes that the future Article 4b (2) and (3) of the modified law of August 1, 2018, on the mandatory declaration of certain diseases complies with the aforementioned requirements. For legal certainty purposes, it advises the authors of the draft law to specify in more detail in the body of the text which data or categories of data are covered by the terms "patient administrative data" and "administrative data of the vaccinating healthcare provider" as provided in points 1° and 4° of paragraph (3).

5. Nevertheless, the CNPD has difficulties understanding the concrete process of creating the digital record and has practical questions regarding the initial processing of the personal data included therein. Indeed, it emerges from a combined reading of the future Article 4b (4).

****6.** [Continuation of the document follows.]**

of the amended law of August 1, 2018, on the mandatory declaration of certain diseases and of Article 1 (1) of the draft Grand-Ducal regulation that healthcare providers must record patient data in the digital booklet at the time of vaccination or no later than seventy-two hours afterward, while the creation of the digital booklet by the eHealth Agency only occurs from the first entry by the healthcare provider. The commentary on Article 1 of the draft Grand-Ducal regulation specifies in this context that

in order for the eHealth Agency not to create “[...] indiscriminately an electronic vaccination booklet for any person with a national identification number but without a vaccination, it is indicated, in light of the principles of data minimization and administrative efficiency, to create an electronic vaccination booklet only from the first entry.”

However, how can a healthcare provider already enter the data listed in future Article 4bis (3) of the amended law of August 1, 2018, on the mandatory declaration of certain diseases in the digital booklet at the time of vaccination if the Agency only creates it from the first entry by that same provider? Either the digital booklet has already been created by the eHealth Agency and therefore exists at the time of the first entry by the healthcare provider, or the latter must request the creation of a digital booklet from the eHealth Agency at the time of vaccination, and only afterward can they enter the aforementioned data. The CNPD therefore considers it essential that clarifications on this matter be included in both draft legal texts.

19. Furthermore, the CNPD notes that future Article 46/s (5) of the amended law of August 1, 2018, on the mandatory declaration of certain diseases provides that the data of individuals vaccinated against COVID-19 are transferred to the digital booklet. It understands that this transfer is justified, particularly so that individuals continue to benefit from proof of vaccination in the event of damages or requests for monitoring adverse effects, and allows them to keep track of their vaccination against COVID-19. In this context, the National Commission wonders whether the individuals concerned also have the possibility to request that information on any previous vaccinations contained on their yellow card is similarly transferred to the digital booklet.

20. Finally, as future Article 4bis (4) of the amended law of August 1, 2018, on the mandatory declaration of certain diseases provides that the patient must consent to the recording of data related to future vaccinations other than those recommended by the State, the CNPD wishes to emphasize that all conditions for valid consent in the sense of the law must be met.

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GDPR must be respected, notably that consent must be free, specific, informed, and unambiguous. 18
Regarding access to personal data

21.

According to Article 3.1 of the draft Grand-Ducal Regulation, the digital health record is accessible by the patient, the healthcare provider, as well as the health authority via the electronic platform for the exchange and sharing of health data referred to in Article 60ter of the Social Security Code. It is also stated that the healthcare provider can "also access the vaccination record from a software program compliant with the provisions of Article 11, paragraph (2) and Article 12, except for its paragraph (2), subparagraph 4, point (b), of the amended Grand-Ducal Regulation of December 6, 2019, specifying the modalities and conditions for the establishment of the shared care record." According to the commentary on Article 3 of the draft Grand-Ducal Regulation, the eHealth Agency must ensure in such a case that the intended connection guarantees an appropriate level of security and that the procedure on the interoperability of the software program referred to in Article 12 of the Grand-Ducal Regulation of December 6, 2019, specifying the modalities and conditions for the establishment of the shared care record, is to be applied. The CNPD draws the authors of the draft Grand-Ducal Regulation's attention to the fact that the aforementioned Grand-Ducal Regulation of December 6, 2019, provides for the modalities of security and interoperability in its Articles 10 and 11 and not in Articles 11 and 12 as currently mentioned by Article 3.1 of the draft Grand-Ducal Regulation.

22.

Regarding access to the data contained in the digital health record and subject to its comments in point

12 of this opinion, the CNPD notes that the future Article 46/s (6), second paragraph of the amended law of August 1, 2018, on the mandatory declaration of certain diseases provides that the health authority may access pseudonymized data for all eight purposes stated in paragraph (2) of said article, whereas access to nominative data is limited to only three of the eight purposes.

The CNPD therefore reminds that, in practice, pseudonymization consists of replacing directly identifying data (such as name, first name, or exact address) of a dataset with indirectly identifying data such as an alias or a sequential number. Pseudonymization thus allows for the processing of individuals' data without being able to directly identify them. Unlike anonymization, pseudonymization is a reversible operation. It is therefore possible to indirectly identify a person if additional information is available.

Recital (26) of the GDPR specifies in this context that "personal data that have been pseudonymized and could be attributed to a person..."

18 See definition of consent in Article 4 point 11 of the GDPR, as well as the conditions applicable to consent provided in Article 7 of the GDPR.

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****Physical identification through the use of additional information should be considered as information concerning an identifiable natural person. To determine whether a natural person is identifiable, it is necessary to take into account all means reasonably likely to be used by the data controller or by any other person to identify the natural person directly or indirectly, such as targeting. To establish whether means are reasonably likely to be used to identify a natural person, it is necessary to consider all objective factors, such as the cost of identification and the time required for it, taking into account the technologies available at the time of processing and their evolution.****

23. The CNPD therefore raises the question of who will be responsible for the pseudonymization of the data, and prior to what action. While the data can generally only be pseudonymized after their registration in the digital record, it does not emerge from the texts under review who would be tasked with the pseudonymization. It could be carried out by a "trusted third party," which should be understood as a natural or legal person authorized to perform legal security operations, including pseudonymization or anonymization services, and who presents guarantees of independence, competence, and is not in a conflict of interest regarding the data they process in the context of their various activities.

24. It also questions how it will be practically guaranteed that the health authority will have access to identifiable data for certain purposes, while it will only be able to access pseudonymized data for other purposes, and who will ensure that this limitation is respected.

25. The CNPD notes with satisfaction that Article 4.1 of the draft Grand-Ducal regulation provides for a logging system for access, that is to say, a record of the identification data of the person who consulted or recorded one or more data in the digital record, as well as the context of their intervention.

According to this article, "logging data follows the same lifecycle as the data to which it relates," meaning that the retention period for logs is the same as that of the data in the digital record. As the future Article 4bis (8) of the amended law of August 1, 2018, on the mandatory declaration of certain diseases provides that the data contained in the digital record is retained for thirty years following its encoding by the healthcare provider, it follows that the logging data would also be retained for thirty years.

26. Nevertheless, while logging files can be considered as an element serving to fulfill the security obligation arising from Article 32 of the GDPR by preventing unauthorized access to personal data, the data in these files that allow for the identification of individuals must, like all personal data, ****[Note: The**

text appears to be incomplete here.]**

See commentary on the articles of the draft Grand-Ducal regulation, ad article 4.

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personnel data shall not be retained in a form allowing the identification of the concerned individuals for a period exceeding that necessary in relation to the purposes for which they are processed in accordance with Article 5.1.e) of the GDPR.

27.

Log files may also serve the purpose of ensuring the right of access of the concerned individual to personal data relating to them, and more particularly to data concerning the recipients of the data in accordance with Article 15.1.e) of the GDPR. In particular, they may serve the purpose of allowing the tracing of who has accessed personal data. For this type of log files, one of the criteria to be considered is indeed the retention period of the data to which the log file data relates, that is to say, the data whose communications to other persons are traced by the relevant log files.

However, the Court of Justice of the European Union has stated that "the retention period of the basic data may constitute a useful parameter but is not determinative." Furthermore, it has decided that "regulation limiting the retention of information on the recipients or categories of recipients of the data and the content of the data transmitted to a period of one year and consequently limiting access to this information, while the basic data is retained for much longer, cannot constitute a fair balance of the interests and obligations at stake, unless it is demonstrated that a longer retention of this information would constitute an excessive burden for the data controller."

28.

Based on the above, the CNPD wishes to reiterate its recommendation to retain log files for a period of five years, as this period also corresponds to the limitation period for tort claims (such as the violation of professional secrecy sanctioned by Article 458 of the Penal Code), unless a control procedure is underway.

29.

Finally, it notes that the commentary on the articles of the draft law provides the following: "Regarding pseudonymized data and particularly in the context of monitoring and evaluating the vaccination coverage of the effectiveness of vaccines recommended by the State, access should allow for cross-referencing with other data sources, for example, socio-demographic data, in order to obtain the most relevant results that would allow for the necessary measures to protect the most vulnerable populations." Without further details, the National Commission therefore wonders what these "other data sources" are, on the one hand, and whether cross-referencing with personal data contained in other state files is planned, on the other hand.

20 CJEU, judgment of May 7, 2009, C-553/07, EU:C:2009:293, paragraphs 58 and 66.

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IV.

Regarding the right to object

30.

It follows from the future Article 4bis (7) of the amended law of August 1, 2018, on the mandatory declaration of certain diseases that neither patients nor healthcare providers can oppose the recording of their data in the digital health record, on the one hand, and that they are informed of this limitation prior to the first recording of their data, on the other hand. Article 1 (2) of the draft Grand-Ducal regulation states in this context that it is the responsibility of the healthcare provider to provide the patient, before the creation of their digital health record, "the information regarding the processing of personal data and in particular regarding the limitation of the right to object." The commentary on Article 2 of the draft law contains justifications for the necessity of limiting the right to object of the concerned individuals "in order to achieve the public health purposes for which the electronic vaccination record is established and in particular to determine the vaccination coverage rate as well as to ensure traceability and manage the stocks of vaccines distributed by the State." It is also specified that it is "in the interest of public health to limit the individual right of persons to oppose the recording of their personal data" and that such "a measure is necessary and proportionate because without it the benefits of the [digital] record cannot be realized."

31.

The CNPD wishes to emphasize that, in principle, every concerned individual has the right to object "at any time, for reasons relating to their particular situation, to the processing of personal data concerning them based on Article 6, paragraph 1, point e) or f), including profiling based on these provisions," in accordance with Article 21 of the GDPR. In this case, "the data controller shall no longer process the personal data unless it demonstrates that there are legitimate and compelling grounds for the processing which override the interests, rights, and freedoms of the concerned individual, or for the establishment, exercise, or defense of legal claims."

32.

Limitations on the scope of this right may be provided for by European Union law or the law of a Member State through legislative measures based on Article 23 of the GDPR. However, said article provides in its paragraph 1 that the rights of the concerned individual may be limited when such limitation respects the essence of the freedoms and rights.

21 See Article 21.1, second sentence of the GDPR.

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fundamentals and that it constitutes a necessary and proportionate measure in a democratic society to guarantee the objectives outlined in this paragraph. Furthermore, it should be noted that when such a restriction on individuals' rights is provided for by a legislative measure, it must contain specific provisions regarding the information listed in paragraph 2 of Article 23 of the GDPR.

33. Therefore, it is necessary to verify the compliance of the limitation of the right to object provided for by the future Article 46/s (7) of the amended law of August 1, 2018, on the mandatory declaration of certain diseases with the aforementioned requirements of Article 23 of the GDPR.

34. The EDPS has clarified that the Court of Justice of the European Union and the European Court of Human Rights require that "legislative measures aimed at limiting the scope of the rights of data subjects or the obligations of the data controller must be foreseeable for the data subjects." Similarly, the analysis and finding of necessity and proportionality of a limitation based on a legislative measure should be carried out by the legislator before deciding to impose a restriction. Furthermore, "[t]he link between the limitations provided and the objective pursued should be clearly indicated in the legislative measure." This link must therefore be expressly specified.

35. The CNPD notes that according to the authors of the draft law and their commentary on the

articles, the measure limiting the right to object of the data subjects constitutes a necessary and proportionate measure to ensure the achievement of public health purposes for which the digital record is established, particularly the determination of vaccination coverage rates, as well as the traceability and management of vaccine stocks distributed by the State. It therefore understands that this limitation could aim to guarantee an important public interest objective in the field of public health in accordance with Article 23.1.e) of the GDPR. However, the National Commission finds that the explanations on this matter in the commentary on the articles of the draft law are not sufficient, as they are not included in the draft law itself and will not allow the data subject to understand the link between the limitation of the right to object and the safeguarding of public interests in the field of public health.

22 These include the purposes of processing; categories of personal data; the scope of the introduced limitations; safeguards to prevent abuse or unlawful access or transfer; the determination of the data controller; retention periods and applicable safeguards; risks to the rights and freedoms of data subjects and the right of data subjects to be informed of the limitation, unless this risks undermining the purpose of the limitation.

23 EDPS Guidelines on limitations under Article 23 of the GDPR, point 17.

24 EDPS Guidelines on limitations under Article 23 of the GDPR, point 40.

25 EDPS Guidelines on limitations under Article 23 of the GDPR, point 21.

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once the text is adopted. In particular, it is necessary to specify in the text of the draft law how the limitation of the right to object would contribute to ensuring public health. In accordance with Recital 8 of the GDPR, the reason for the limitation should indeed be understandable to the individuals to whom this limitation applies.

36. Regarding the purposes of the processing, it is necessary to clarify why the right of the data subject to object is limited (and not the purposes of the processing carried out in general). The provision under review must be understandable to the individuals to whom it applies, which implies a clear understanding of how and when the limitation applies. Similarly, Article 23.2.g) of the GDPR requires that the legislative measure takes into account the risks that the limitations pose to the rights and freedoms of the data subject. This is a very important legislative textual clarification, which contributes to the assessment of the necessity and proportionality of the limitations. Clarifications on these two points should therefore be integrated into the body of the text of the draft law.

37. Finally, the EDPS is of the opinion that "a general exclusion of the rights of data subjects with respect to all or certain data processing operations or with respect to certain data controllers would not respect the essence of the fundamental right to the protection of personal data as enshrined in the Charter." The French counterpart of the CNPD, the National Commission for Information Technology and Civil Liberties (CNIL), has also stated that "the limitations provided by law must therefore be proportionate and must not generally undermine the rights guaranteed by the GDPR. In this regard, the following should be prohibited: the exclusion, for the same processing, of all the rights provided by the GDPR as well as the general exclusion of the right to object for all processing carried out by a public authority."

38. The National Commission can only align itself with these findings. Therefore, it is essential that all other rights conferred to data subjects by Articles 12 to 20 and 22 of the GDPR are fully respected by the various data controllers involved, namely by the eHealth Agency, the health authority, as well as the various healthcare providers. Furthermore, the limitation of the right to object cannot solely target the processing of personal data carried out through the digital health record and in no case other

processing carried out by the said data controllers.

Thus adopted in Belvaux on February 23, 2024.

The National Commission for the Protection of Data

Tine A. Larsen

President

Thierry Lallemand

Commissioner

Marc Lemmer

Commissioner

Alain Herrmann

Commissioner

CNPD

Opinion of the National Commission for the Protection of Data regarding draft law No. 8331 amending the amended law of August 1, 2018, on the mandatory declaration of certain diseases within the framework of public health protection and regarding the draft grand-ducal regulation specifying the modalities and conditions for the establishment of the electronic vaccination record.

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